

# **Prior Authorization Request**

## **Health Plan**

Humana Medicare Advantage -- Gold Choice PPO.

## **Member Information**

Member: Gordon W. Ashcombe DOB: 1957-07-19 (age 69) Member ID: MBR-64412

Sex: M

## **Requesting Provider**

Practice: Meridian Valley Cardiology

## **Requested Service**

Requested service: Elective inpatient admission, implantation of a transvenous implantable cardioverter defibrillator (ICD)

CPT 33249

Planned admit date: 2026-11-09

Level of care requested: INPATIENT

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## History and physical

MERIDIAN VALLEY CARDIOLOGY - H&P

DOS: 09/16/2026 Ashcombe, Gordon W. 69 M

CC: ICD eval

PMH:

- Anterior STEMI 03/2014 -> PCI/DES to prox LAD 03/11/2014
- Ischemic cardiomyopathy
- COPD (former smoker 40 pk-yr, quit 2014)
- DM2, diet + metformin
- CKD 3a

FHx: Father - MI at 52, CABG at 60, died 71. Mother - DM2, d. 84. Brother (63) - Brugada syndrome, has ICD since 2019. Sister healthy.

SHx: retired electrician. Lives w/ wife. EtOH rare.

ROS: + DOE (1-2 blocks), - CP, - syncope, - palpitations, - orthopnea.

Exam: BP 118/70, HR 66. Mild expiratory wheeze. No edema.

A/P: ICM, NYHA class II, LVEF 29% (TTE 09/03). No ACS or repeat revascularization since 2014. Brugada evaluation completed given brother's dx (see EP note) - negative. Refer for primary prevention ICD.

W. Pritchard-Lowe, MD

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## Echocardiogram report

Meridian Valley Cardiology - TTE

Date: 09/03/2026 Patient: ASHCOMBE, GORDON

LVEF 29% (biplane Simpson)

Anterior, anteroseptal and apical akinesis, thinned/scarred.

LVIDd 6.0. Mild MR. RVSP 30.

Impression: Severe ischemic LV dysfunction, LVEF 29%.

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## EP note - Brugada evaluation

MERIDIAN VALLEY - ELECTROPHYSIOLOGY

Procainamide challenge 08/18/2026

Indication: brother with Brugada syndrome; screen pt before device planning

Baseline ECG with high precordial leads (V1-V2 at 2nd/3rd ICS): no type 1 pattern.

Procainamide 10 mg/kg IV: no coved ST elevation, no type 1 pattern. QRS 104 -> 116.

Result: NEGATIVE for Brugada pattern.

Conclusion: patient does not have Brugada syndrome. ICD indication, if any, is based on his ischemic cardiomyopathy.

E. Svensson-Achar, MD

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## Shared decision making note

SDM VISIT - ICD - Meridian Valley Device Clinic

Date 09/30/2026

Gordon Ashcombe, wife present

Clinician: Harriet Moyo, NP

Decision aid: evidence-based ICD decision aid, reviewed (paper + video).

Pt has seen his brother's device and shocks; we separated his brother's situation (inherited rhythm disorder) from his own (scar-related risk).

Risks/benefits/alternatives reviewed. Pt chooses ICD.

H. Moyo NP

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Medication list

Ashcombe, Gordon W. - Current meds (09/16/2026)

- carvedilol 25 mg BID ..... 03/13/2014
- sacubitril/valsartan 49/51 BID ..... 05/2020 (lisinopril 03/13/2014-05/2020)
- spironolactone 25 mg ..... 2015
- dapagliflozin 10 mg ..... 2022
- aspirin 81 mg ..... 2014
- atorvastatin 80 mg ..... 2014
- metformin 500 mg BID ..... 2018
- budesonide/formoterol inh BID ..... 2019

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Laboratory results

MV LAB 09/17/2026 ASHCOMBE G

Cr 1.4 eGFR 53 K 4.6 Na 139

A1c 6.9

Hgb 14.4 Plt 210

NT-proBNP 920

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## Pre-admission plan

PRE-ADMISSION / LEVEL OF CARE - EP

10/02/2026 Ashcombe, Gordon CPT 33249 Admit 11/09/2026

Planned stay: 2 midnights (inpatient).

Factors: COPD w/ wheeze, risk of post-sedation bronchospasm/hypoxia; CKD 3a w/ contrast; DM2.

Exclusions: cognitively intact; no illness limiting survival <1 yr; no AF; clinically stable. None.

E. Svensson-Achar MD

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